

--SUMMARY--

Decision No. 934/23

29-Apr-2024

G.McCaffrey - D.Thomson - C.Salama (PT)

- Aggravation (preexisting condition) (osteoarthritis) (knee)
- Permanent impairment {NEL}
- Strains and sprains (knee)

No Summary Available

9 Pages

References: Act Citation

- WSIA

Other Case Reference

- [w2524n]

Style of Cause:
Neutral Citation: 2024 ONWSIAT 689



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 934/23

BEFORE:

G. McCaffrey: Vice-Chair
D. Thomson: Member Representative of Employers
C. Salama: Member Representative of Workers

HEARING:

July 4, 2023 at Toronto
Oral by Videoconference
Post-hearing activity completed on March 25, 2024

DATE OF DECISION:

April 29, 2024

NEUTRAL CITATION:

2024 ONWSIAT 689

DECISION UNDER APPEAL:

WSIB Appeals Resolution Officer (ARO) K. McDonnell dated
June 1, 2022

APPEARANCES:**For the worker:**

B. Leila, Lawyer

For the employer:

Not participating

Interpreter:

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REASONS

(i) Introduction and issue

[1] The worker appeals the decision of Appeals Resolution Officer (ARO) K. McDonnell dated June 1, 2022.

[2] The worker seeks recognition of a permanent impairment (PI) and entitlement to a Non-Economic Loss (NEL) determination for the bilateral knees.

(ii) Background

[3] The following are the basic facts.

[4] The worker began her employment at a packaging company in 2014. On January 14, 2016, at age 57, she slipped on a wet floor in the plant and hurt her knees. She saw a physician that day and was diagnosed with bilateral knee pain. That physician also noted a pre-existing “old knee injury.”

[5] The worker was placed on modified work and on January 27, 2016 began physiotherapy treatment.

[6] The claim was accepted for health care benefits for bilateral knee strains.

[7] A March 17, 2016, right knee MRI that showed osteoarthritis, meniscal tears, near-complete cartilage loss, and large osteophytes.

[8] The April 15, 2016 physiotherapy discharge report indicated the worker was pain-free with strong right knee ranges of motion, and could return to her regular duties and hours.

[9] There was no further recorded contact from the worker until 2021 when she claimed entitlement for bilateral knee permanent impairments. Medical reporting in 2021 indicated the worker had severe degenerative osteoarthritic changes in both knees that limited her mobility, and she had difficulty standing, walking, sitting, kneeling, climbing stairs, lifting, and carrying.

[10] On November 8, 2021 the WSIB found the March 2016 MRI findings were in keeping with the worker's age-related degenerative changes that were pre-existing and unrelated to the work injury. The WSIB found the worker had fully recovered from the bilateral knee strains, returning to her pre-injury abilities. The ARO decision of June 1, 2022 confirmed the denial on ongoing entitlement, finding that the worker had recovered from the minor work-related knee strains by April 15, 2016, and that there was no evidence that the January 2016 slip at work caused or aggravated the MRI findings.

(iii) Law and policy

[11] As the worker was injured in 2016, the *Workplace Safety and Insurance Act, 1997* (the WSIA) is applicable to this appeal. All statutory references in this decision are to the WSIA, as amended, unless otherwise stated.

[12] Section 46 of the WSIA and section 42 of the pre-1997 *Workers' Compensation Act*, as amended, provide that if a worker's injury results in permanent impairment, the worker is entitled to compensation for non-economic loss.

[13] “Impairment” means a physical or functional abnormality or loss (including disfigurement) which results from an injury and any psychological damage arising from the abnormality or loss.

[14] “Permanent impairment” means impairment that continues to exist after the worker reaches maximum medical recovery.

[15] Tribunal jurisprudence applies the test of significant contribution to questions of causation. A significant contributing factor is one of considerable effect or importance. It need not be the sole contributing factor. See, for example, *Decision No. 280*, 1987 CanLII 1996 (ON WSIAT). The standard of proof in workers’ compensation proceedings is the balance of probabilities.

[16] We have considered the policies the WSIB stated apply to the subject matter of this appeal, pursuant to section 126 of the WSIA.

[17] *Operational Policy Manual* (OPM) Document No. 11-01-05, “Determining Permanent Impairment” indicates in part:

Determining ongoing impairment

Once MMR has been determined, decision-makers consider whether there is an ongoing impairment based on the clinical evidence.

The WSIB considers

- a physical abnormality to be a change to or damage to a body part or organ system,
- a physical loss to be a loss of some or all of a body part or organ system,
- a functional abnormality to be a malfunction of a body part or organ system,
- a functional loss to be a loss of some or all of the functioning of a body part or organ system,
- a disfigurement to be an altered or abnormal appearance such as an alteration of color, shape, structure, or a combination of these, and
- psychological damage to be the loss of or abnormal psychological functioning.

Factors such as the type or duration of treatment are not generally considered indicators of ongoing impairment in the absence of other clinical evidence of impairment.

Determining work-relatedness

The decision-maker must confirm that the ongoing impairment is work-related by considering

- whether the current diagnosis is the same as or compatible with the initial work-related injury/disease diagnosis,
- whether the clinical evidence of impairment is related to the current diagnosis, and
- whether a pre-existing condition or other non-work-related factor is causing or contributing to the impairment.

If the work-related injury/disease and a pre-existing condition or non-work-related factor are both contributing to the degree of total impairment to the area, the impairment attributable to the work-related injury/disease is determined.

If the ongoing impairment is caused solely by a pre-existing condition and/or non-work-related factor, there is no permanent impairment.

(iv) Submissions

[18] At the July 4, 2020³ hearing the worker representative submitted that although the original reports do not specifically state the worker fell to the floor, based on her testimony regarding the mechanics of the injury, and that two workers helped her up from the floor, it should be accepted that the worker struck both her knees on the floor on January 14, 2016. The worker representative submits that the degenerative changes in the worker's knees identified in 2010 were consistent with her years of working as a labourer, her age, size, and weight, and there is no evidence that the worker's knees were symptomatic prior to the slip and fall at work.

[19] The worker representative submits that the worker never recovered from the work accident, as evidenced by the fact that she had to stop working in 2018, and Dr. Saito's November 23, 2021 report outlining the worker's continuing symptoms.

[20] The worker representative submits that the work accident either caused bilateral knee injuries or permanently aggravated the underlying knee condition. He refers to OPM Document No. 15-02-04 "Aggravation Basis."

[21] The worker representative submits that the worker's ongoing bilateral knee symptoms are work-related and as result a permanent impairment should be recognized.

[22] Following the hearing the Panel requested that the worker's pre accident and post accident medical records be obtained and be shared with the worker representative. No final post hearing submissions were received from the worker representative.

(v) Analysis

[23] We find that the evidence does not support ongoing bilateral knee entitlement and a NEL determination.

[24] The worker testified that she did not have any injuries to either of her knees prior to the January 14, 2016 work injury in this appeal. The worker testified that prior to January 14, 2016 she had not experienced any pain in her knees, sought medical attention for her knees, or received any treatment for her knees.

[25] The worker stated that around 2009 she incurred injuries to her neck and lower back as a result of a motor vehicle accident (MVA). She testified that her knees were not injured.

[26] The worker stated that she had worked primarily as a general labourer in factories, and sometimes since around 2011 worked at a second job as a personal support worker (PSW). She indicated that she had attended school for about six months in 2009 to obtain her PSW certification and had started a part-time second job as a PSW around 2011. She stated the PSW work involved in-home care and accompanying clients to appointments. The worker denied experiencing any knee pain at any of her jobs before January 14, 2016.

[27] When directed to an April 28, 2010 right knee x-ray report, a July 9, 2010 left knee x-ray report, and a July 12, 2010 ultrasound report for both knees in the case materials, the worker testified that her family physician, Dr. R. Saito, arranged for that imaging as part of a general check-up, she was not experiencing any knee symptoms at that time, and Dr. Saito did not

discuss the imaging results with her. The worker also testified that she did not have any knee symptoms between 2010 and the January 2016 slip at work.

[28] The worker stated that her employer helped her complete the Worker's Report of Injury/Disease (Form 6), she did not write "arthritis" on the Form 6, and it is not in her handwriting. We observe that the Employer's Report of Injury/Disease (Form 7) dated January 19, 2016 indicates the worker had pre-existing "arthritis on her knees which may impact her recovery."

[29] The worker stated that as Dr. Saito was not available on January 14, 2016 when she slipped at work, she visited Dr. P. Nizran, family physician, at a different medical clinic. The worker could not explain why Dr. Nizran's Health Professional's Report (Form 8) dated January 14, 2016 indicates "she has pre-existing old knee injury," as she testified that was the first time she had met Dr. Nizran and she did not tell him that she had a prior knee injury.

[30] We find that the worker's testimony that she did not have knee symptoms or injuries prior to the January 14, 2016 work injury is not consistent with the contemporaneous medical evidence. Based on the medical documentation received post hearing, we find that the worker is at best a poor historian, and that we cannot rely on her testimony.

[31] The medical evidence now in the case materials begins in April 2010. The MVA referred to by the worker occurred on April 12, 2010. The MVA related reporting confirms the worker's testimony that she was a passenger in a vehicle that was rear ended. In addition to the soft tissue injuries to the neck and low back that the worker testified to, it was reported the worker bruised her right knee when she struck it against the driver's seat in front of her. The April 28, 2010 x-ray of the right knee identified "mild osteoarthritic changes."

[32] The earliest reference to the left knee is on July 5, 2010, when the worker reported to Dr. Saito that she fell onto her left knee at a grocery store on July 3, 2010. A July 9, 2010 x-ray of the left knee identified "mild degenerative change in the medical aspect of the knee joint." The worker was then saw Dr. Saito multiple times for left knee pain until February 2011, indicating that the left knee was symptomatic for at least six months following the July 2010 fall.

[33] A July 28, 2011 Insurer's Examination Occupational Therapy In-Home Assessment by H. Babani, which relates to the 2010 MVA, indicated the worker continued to report intermittent right knee pain. An August 25, 2011 independent medical exam (IME) by Dr. G. Moddal, neurologist, related to the 2010 MVA, indicated that the worker had not returned to work. An August 26 2011 IME by Dr. A. Czok, physiatrist, related to the 2010 MVA, noted the worker has "crepitus of the patellae bilaterally, right more than left."

[34] The clinical notes of Dr. Saito then contain subsequent references to the knees. On December 13, 2013 the worker reported right knee pain. On July 21, 2014 the worker reported "a recurrence of pain in the right knee, this is since her car accident." There were right knee complaints again in August 2014 and October 2014.

[35] A February 3, 2016 clinical note of Dr. P. Nizran, family physician, indicates the worker had a cortisone injection in her right knee in November 2015, which was two months before the fall at work in this appeal. A February 3, 2016 ultrasound of the right knee for "right knee pain and swelling, chronic," refers to a prior x-ray on November 23, 2015, and an ultrasound on March 21, 2013, which are not contained in the case materials.

- [36] In short, we find that the objective evidence supports that the worker had knee symptoms since at least April 2010, that the imaging identifies well established degenerative changes in both knees before the January 2016 work injury, and the November 2015 right knee cortisone injection is an indication the right knee was symptomatic in the months before the January 2016 work injury.
- [37] Turning to the mechanism of injury on January 14, 2016, the original reporting indicates the worker slipped on a wet floor at work. The worker testified that she was not carrying anything, the spilled product on the floor that she slipped on was cream, and she fell straight forward, landing on both knees. She testified that she felt pain only on the front of her knees. The worker described the fall in detail, did not give any indication that there was a twisting motion, but stated she fell straight forward. She stated the two co-workers listed on the Form 6 helped her up from the floor.
- [38] The worker testified that she returned to modified work after three days. She stated that her regular work involved rotating between a number of machines and packaging. She indicated that rotation included working on a machine that required the operator to stand, and before the accident she operated that machine for a couple of hours, one to two days each week. The worker testified that after her bilateral knee injury she never worked on the machine that required the operator to stand again.
- [39] The worker testified that she undertook physiotherapy for about six weeks following the work injury. The case materials indicate that physiotherapy treatment began on January 27, 2016 and ended on April 13, 2016. The worker stated that she had no subsequent physiotherapy treatment.
- [40] We note from Dr. Nizran's clinical notes that the worker received a right knee injection on March 17, 2016. There was then no reference to the knees on the next three visits, and the April 20, 2016 note indicates "knees are better, since she got her steroid she is feeling better, minimal crepitus in bilateral knees no swelling no tenderness in the joint line, knee pain improved."
- [41] The worker stated that she had no further treatment for her knees after physiotherapy ended in April 2016. With respect to medication for her knees, the worker testified that she would take two Advil tablets when needed.
- [42] There had been no reference to the left knee in the clinical notes since February 12, 2016, which was approximately one month after the fall at work. After April 20, 2016 the worker saw Dr. Nizran 14 times before there was another reference to her right knee. Then on September 8, 2016 the worker received a repeat cortisone injection in the right knee. The clinical note indicates "history of knee osteoarthritis," and "tender medical joint and crepitus on knee flexion."
- [43] Therefore, we find that by April 20, 2016 the worker's work-related bilateral knee strains had resolved based on the contemporaneous evidence. There is no reference in the clinical notes to the left knee after February 12, 2016, physiotherapy treatment for the bilateral knee strain had concluded on April 13, 2016, the worker then had no continuing active treatment, and on April 20, 2016 Dr. Nizran indicated that the "knees are better" was consistent with the physical examination on that date.

[44] The worker indicated that prior to the accident she had also been working part-time as a PSW. She testified that after the January 2016 fall at work she never returned to work as a PSW. We were not taken to any contemporaneous reference to the PSW job in the case materials, or that the bilateral knee strain prevented the worker from continuing to work as a PSW in 2016.

[45] With respect to the subsequent knee symptoms, that resulted in a December 2022 total right knee replacement, the worker stated that her knee symptoms worsened and she stopped working in 2018 because of her bilateral knee pain, which she experienced both sitting and standing. The worker's recollection was unclear, but she thought she may have seen Dr. Nizran a number of times before returning to Dr. Saito's care, once he returned from his absence. Dr. Nizran's notes do not refer to the knees during five visits between September 8, 2016 and April 4, 2017, when it was noted "6 mos since last cortisone shot," and the worker received a cortisone injection in her knee for osteoarthritis. There is no indication of further medical attention until January 2018.

[46] Dr. Saito's clinical notes resume in January 16, 2018. We note that a December 12, 2018 short-term disability form completed by Dr. Saito indicates the worker was not working due to a right knee torn medial meniscus, low back pain, and insomnia.

[47] The worker testified that Dr. Saito referred her for the June 2022 bilateral knee injections referred to in the case materials. She stated the injections helped her left knee, but not her right knee, and she had a total right knee replacement on December 21, 2022.

[48] We have found above that the accepted bilateral knee strains of January 14, 2016 resolved by April 30, 2016. We further find that the evidence does not support the worker representative's alternative submission that the January 14, 2016 accident permanently aggravated the underlying knee osteoarthritis.

[49] We note that degeneration in both knees was identified through the 2010 imaging referred to above. We find the November 2015 cortisone injection supports that the right knee continued to be symptomatic just prior to the work accident in January 2016. As noted above, the worker's testimony regarding the mechanics of the workplace fall are not compatible with a turning or twisting injury, as she fell forward onto both knees.

[50] A March 17, 2016, right knee MRI that showed osteoarthritis most severe in the medial and the patellofemoral compartments compartment.

[51] A June 20, 2018 MRI again identified degenerative changes, and a "a degenerative tear through the body of the posterior home of the medical meniscus."

[52] A September 20, 2022 ultrasound reported "bilateral tricompartmental osteoarthritis which is severe in the medial tibiofemoral compartments, worse on the right than the left."

[53] Then on December 21, 2022 the worker had a total right knee replacement. The surgeon, Dr. R. Abughaduma, confirmed right knee tricompartmental varus osteoarthritis.

[54] The July 12, 2021 report of Dr. R. Saito, family physician, attributes the worker's bilateral knees symptoms to severe degenerative arthritis:

This unfortunate lady is suffering from a combination of multiple impairments in both her knees as diagnosed in several tests conducted. Her symptoms and signs are significant. Her reports demonstrated severe degenerative, osteoarthritic changes in both her knees, limiting her mobility.

[55] In a subsequent, almost identical, report dated November 23, 2021, Dr. Saito added:
This unfortunate lady continues to suffer from the injuries she sustained at her workplace.

[56] Dr. Saito provides no explanation why he altered his report to indicate the January 2016 injuries had not resolved.

[57] We assign little weight to the Dr. Saito's apparently altered opinion regarding the worker's ongoing knee symptoms as the November 23, 2021 report does not refer to the worker's pre-existing knee condition, the April 2010 MVA and right knee symptoms, the July 2010 left knee injury and subsequent symptoms, or that the worker had a November 2015 cortisone injection in her knee just prior to the January 2016 fall at work. We also note Dr. Saito was not the worker's treating physician between January 2016 and January 2018. Dr. Saito also does not explain how "multiple impairments in both her knees" arise from a single slip at work, or account for the April 30, 2016 examination which supports that the workplace strains had resolved, or the subsequent gap in continuity of reported symptoms.

[58] Lastly, there is no orthopedic opinion before us that includes the worker's complete history of knee symptoms and attributes the progression in the worker's osteoarthritic knee condition to the January 2016 slip at work in a causative manner.

[59] For all the above reasons we find on the balance of probabilities that the evidence does not support entitlement for the January 14, 2016 bilateral knee injuries beyond April 30, 2016. As there is no ongoing bilateral knee entitlement the worker does not have entitlement for a permanent impairment arising from the January 14, 2016 accident and is not entitled to a NEL determination for the knees.

DISPOSITION

[60] The appeal is denied.

DATED: April 29, 2024

SIGNED: G. McCaffrey, D. Thomson, C. Salama